

What Is a Dental AR Report and What Should You Do When Something Looks Off? June 2026

Learn what a dental AR report is and what to do when balances look off. Keep your AR ratio at 1.0 and under 5% past 90 days. June 2026 best practices.

Max Shore – July 6, 2026

Every dental practice produces more dollars than it collects in any given week, and the gap between what you have earned and what has actually landed in your account is where cash flow problems hide. A dental AR report is the document that makes that gap visible, breaking down exactly who owes you, how much, and how long the money has been sitting unpaid. This guide walks through what an AR report shows, how to read it, the red flags worth watching for, and what to do when the numbers start drifting in the wrong direction.

TLDR:

A dental AR report tracks every dollar owed to your practice, organized by age and payer type. Keep your AR ratio at 1.0 or below, with under 5% of balances sitting past 90 days. Run AR reports weekly and tackle the 90+ day bucket first to avoid write-offs. Balances past 90 days drop to 15–25% collectability, making early action critical.

What is a Dental AR Report?

A dental AR (accounts receivable) report is a financial tracking document that lists every dollar still owed to your practice, sorted by who owes it and how long it has been outstanding. Patients carry part of that balance. Insurance carriers carry the rest. Together, they form the picture of cash your practice has earned but not yet collected.

The structure follows a standard aging format, with balances bucketed by how many days they have been sitting unpaid:

0 to 30 days: recently billed, typically still within normal payment windows

31 to 60 days: starting to drift, worth a closer look

61 to 90 days: aging into problem territory

90+ days: high risk of becoming uncollectable

Each bucket tells you something different about the health of your billing workflow. A heavy 0 to 30 column usually reflects steady production. A bloated 90+ column points to claims falling through the cracks, patient statements going unanswered, or denials nobody chased down.

Most practice management systems generate this report on demand, and reading it well is one of the clearest ways to gauge how your office is actually performing financially, beyond what the schedule and production numbers suggest.

How to Read Your AR Report

Reading the report well means looking past the total at the bottom and into the relationships between the numbers.

Start with your AR ratio, which compares total accounts receivable to average monthly production. According to dental practice benchmarking research, the long-standing industry target is 1.0, meaning outstanding AR should roughly equal one month of production. A ratio creeping toward 1.5 or 2.0 signals collections are lagging behind production.

Next, look at the distribution across aging buckets. Two benchmarks matter:

No more than 10% of AR should sit past 60 days

Per SmilePass guidance, no more than 5% should sit in the 90+ day bucket

If either threshold gets crossed, something is stuck in the workflow.

AR Aging Bucket	Benchmark Target	What It Signals When Over Target
0 to 30 days	Should hold the majority of total AR balance	Recently billed amounts still within normal payment windows
31 to 60 days	Less than 10% of total AR combined with 61 to 90 bucket	Claims or patient balances starting to drift and need attention
61 to 90 days	Less than 10% of total AR combined with 31 to 60 bucket	Balances aging into problem territory requiring immediate follow-up
90+ days	No more than 5% of total AR	High risk balances with 15 to 25% collection probability
Overall AR Ratio	1.0 or below (total AR equals one month production)	Collections lagging production, cash flow problems mounting

From there, drill into account-level detail. Sort by balance size within each aging bucket so the largest dollars rise to the top. A single \$4,800 claim sitting at 75 days deserves attention before a dozen \$90 patient balances at 35 days. Cross-reference the insurance column against the patient column too, since a balance still attributed to insurance at day 60 usually means a claim was never paid, appealed, or followed up on.

Common Red Flags in Dental AR Reports

A handful of patterns tend to surface when something is wrong, and each one points to a different breakdown somewhere upstream.

Aging concentration past 90 days. When more than 5% of total AR sits in the 90+ bucket, claims are getting abandoned mid-cycle. If you're behind on AR, automation can help catch up. Usually denials no one appealed, or patient balances no one followed up on.

AR ratio above 1.5. According to SmilePass guidance on dental AR, a ratio well above 1.0 means your practice is acting as a bank, extending credit to patients and carriers instead of collecting on production.

Deposit-to-revenue mismatches. Gaps between bank deposits and recorded revenues, unexplained AR adjustments, or daily collection variances can signal fraudulent activity.

Sudden write-off spikes. A jump in adjustments without a matching policy change often masks posting errors, miscoded denials (for example, a CO-4 procedure-modifier mismatch written off instead of corrected and resubmitted), or staff using write-offs to clear balances they cannot resolve.

Stale insurance balances on routine procedures. Cleanings and exams aging past 45 days on the insurance side usually mean a claim was never submitted, rejected silently, or lost in a clearinghouse.

Each pattern points to a different fix, and treating them as one problem is how AR backlogs grow.

What to Do When Your AR Report Shows Problems

Once the red flags are mapped, the work moves to clearing them.

Start with cadence. Run aging reports weekly to catch slippage before balances cross the 60 or 90 day line. A weekly rhythm also gives the front desk time to act on claims that stalled mid-cycle.

From there, work the report in tiers:

Hit the 90+ bucket first. These balances carry the highest write-off risk, so focus calls, statements, and payer follow-ups on the oldest claims before touching anything else.

Separate insurance AR from patient AR. Stalled insurance claims usually need a resubmission, an appeal, or a missing attachment, such as a periapical X-ray on a CO-16 missing-information denial or a perio chart on a scaling and root planing claim. Short-staffed practices can benefit from automated follow-ups. Patient balances need a statement, a payment plan, or a card on file.

Investigate denial codes line by line. A cluster of CO-16 or CO-197 denials points to a coding or preauth issue upstream, not a collections problem.

Review credit balances monthly. Overpayments left sitting on the report inflate AR and create refund liability the longer they age.

Document every touch inside the patient ledger. When the next AR report runs, you want a clear trail showing what was attempted, when, and what the payer or patient said. Ready to clean up your AR? [Get started](#) today.

Why AR Reports Matter in Dental Practices

The AR report isn't a back-office artifact. It's the single document connecting daily clinical production to the cash actually landing in your account, and how you treat it shapes nearly every part of practice performance.

Bottom-line impact. Mastering AR flow is one of the highest-ROI activities a dental practice can do. Ignore it and you run growth-limited at best, cash-deficit at worst.

Collection probability drops fast. According to Pearly's research on AR costs, once a balance ages past 90 days, the chance of collecting falls to 15 to 25 percent. Every week the report goes unread is money slipping toward write-off.

Cash flow stability. Predictable AR means predictable payroll, supply orders, and rent coverage. A volatile aging report forces owners to float operating costs personally.

Workflow visibility. The report exposes where claims stall, which payers run slow, and which front desk processes need tightening. For example, if every Delta Dental claim sits unpaid at day 45 while other carriers clear in 20, the report points you straight to a payer-specific submission or attachment problem instead of a blanket collections push. Without it, you're guessing.

Growth capacity. Practices with clean AR can finance new chairs, hires, or locations on collected revenue instead of debt. Learn more about [reimagining the doctor's office](#).

Final Thoughts on Getting Control of Your Dental AR

Once you understand what the aging buckets mean and which patterns signal real problems, your AR report becomes one of the most actionable documents in your practice. Start with a weekly review cadence, focus your energy on the 90+ day column, and separate insurance follow-up from patient collections so nothing falls through the cracks. The report tells you exactly where your money is stuck, so fixing those leaks is how you stabilize cash flow and stop floating operating costs out of pocket. Want to see how Vendor keeps your AR clean automatically? [Book a demo](#) today.

FAQ

What's the target AR ratio for a dental practice?

The benchmark is 1.0, meaning your total AR should equal roughly one month of production. A ratio above 1.5 signals collections are falling behind, and anything near 2.0 means cash flow problems are likely mounting.

How do I know if my AR report is showing red flags?

Look for three warning signs: more than 5% of total AR sitting past 90 days, insurance balances on routine cleanings aging past 45 days, or sudden spikes in write-offs without a policy change. Each pattern points to claims stalling at different stages of your billing workflow.

AR report Vendor vs full-service billing agency?

Vendor automates the posting and reconciliation work with software, flagging AR issues in real time but leaving follow-up calls to your team. Full-service agencies handle claims submission, aging follow-up, and appeals with human staff; work Vendor doesn't cover yet. If your AR problems stem from missed posts or reconciliation errors, Vendor fixes that. If they stem from lack of payer follow-up capacity, you need human support.

How often should I run my dental AR aging report?

Weekly. Running it monthly lets balances drift past 60 or 90 days before you catch them, and collection probability drops to 15 to 25% once claims age past 90 days. A weekly cadence gives your front desk time to act before money slips toward write-off.